



AAMS Fundamentals: Module 1

Welcome to All American Medical Staffing. This comprehensive orientation establishes the foundation for clinical excellence, professional conduct, and survey-ready practice that defines every AAMS nurse.

Mission, Vision & Core Values

Our Mission

To deliver safe, compliant, exceptional patient-centered care by empowering clinicians with the tools, training, and support they need to excel in every home health environment.

Our Vision

To be recognized nationwide as the most dependable, survey-ready, and clinically excellent healthcare staffing partner—where nurses are known for superior skill, compassion, and professionalism.



Core Values in Action



Integrity

Completing every visit honestly and accurately, never falsifying documentation or cutting corners on patient safety.



Accountability

Submitting documentation same day and communicating delays before they occur to maintain seamless care coordination.



Excellence

Providing high-quality assessments, thorough teaching, and maintaining professional presence in every patient interaction.



Compassion

Treating every patient with dignity, patience, and respect regardless of circumstances or challenges encountered.



Reliability

Showing up on time, meeting deadlines, and maintaining consistent communication with all stakeholders.



Continuous Improvement

Seeking feedback, learning new clinical changes, and constantly refining practice to deliver better patient outcomes.

What It Means to Be an All-American Nurse

Being an "All American Nurse" is not just a job—it's a standard of excellence. You represent three critical entities simultaneously: the agency you are assigned to, AAMS as your employer, and your own professional nursing license. Each visit, each interaction, and each documentation entry reflects on all three.

01

Commitment to Compliance

Following all state, federal, and agency requirements while maintaining survey-ready documentation and practicing within your scope of practice.

03

Reliability & Performance

Showing up on time, visiting patients within scheduled windows, and submitting documentation the same day without exception.

02

Patient -Centered Care

Delivering respectful, culturally sensitive, safe care while maintaining professionalism in all interactions and prioritizing safety, quality, and dignity.

04

Adaptability

Recognizing that every home, patient, and agency operates differently, requiring quick professional adaptation to diverse environments.

All American Nurse Behavior Standards

Expected Behaviors

- Answering agency calls promptly and professionally
- Calling ahead before every scheduled visit
- Reporting red flags immediately to appropriate parties
- Documenting teaching methods and patient response thoroughly
- Maintaining a polished, professional bedside manner
- Coordinating care seamlessly with interdisciplinary teams

Unacceptable Behaviors

- No-showing or visiting outside ordered frequencies
- Submitting vague or incomplete documentation notes
- Engaging in arguments with patients or family members
- Using unprofessional language or tone in any setting
- Ignoring communication from agencies or AAMS staff
- Failing to escalate critical patient changes

Agency Partnership Expectations

AAMS maintains professional contracts with some of the most established home health agencies in the nation. To protect those partnerships and ensure patient safety, our expectations are clear and non-negotiable. Your performance directly impacts contract renewals and future opportunities.

1

Professionalism

Demonstrate punctuality and consistency in scheduled visits while maintaining respectful communication with staff, patients, and families. Represent AAMS positively at all times.

2

Communication Standards

Confirm visits with patients, notify agencies of delays or cancellations, escalate clinical concerns immediately, and respond to AAMS messages within expected timeframes.

3

Documentation Excellence

Submit accurate, complete notes same day. Document all teaching, interventions, and patient responses. Ensure notes match the POC and respond to QA corrections within 24 hours.

4

Safety & Compliance

Maintain infection control standards, follow state and federal guidelines, maintain current licensure and certifications, and report exposure incidents immediately.

Professional Standards & Brand Representation

AAMS nurses are ambassadors of the organization. Everything you do—from your physical appearance to your communication style—reflects on the company and impacts future contracts. Your conduct maintains and elevates our reputation for excellence.



Physical Presentation

Clean scrubs, minimal fragrance, professional hygiene, and visible ID badge at all times.



Communication Style

Clear, respectful speech avoiding slang. Document factually and objectively while maintaining composure.



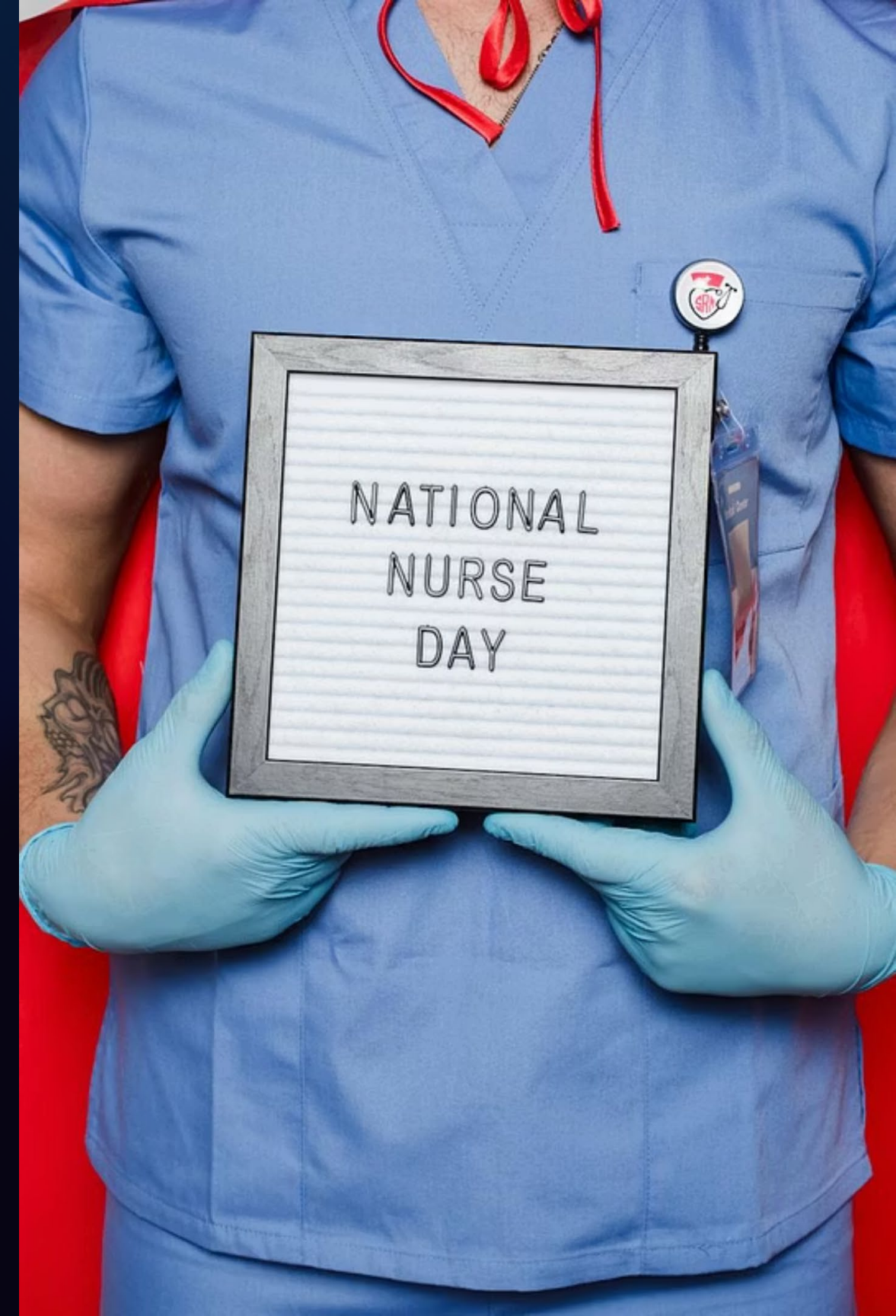
Professional Boundaries

Maintain patient privacy, keep professional boundaries, and stay focused on clinical tasks.



AAMS Brand Identity

We are known for survey-ready documentation, highly skilled clinicians, strong professionalism, and reliable follow-through. Your conduct directly impacts this reputation.



The AAMS Standard of Excellence Framework

To ensure every nurse delivers consistent, high-level care, AAMS uses a comprehensive six-pillar framework that guides practice from pre-visit preparation through post-visit documentation and communication.

Preparedness

Review POC, medications, and prior notes before visits. Bring necessary equipment and PPE. Verify all orders.

Reliability

Meet visit expectations consistently. Respond promptly to all communications and maintain compliance.

Communication Excellence

Notify agencies of critical changes. Provide clear, concise updates and communicate barriers promptly.



Presence

Arrive on time with a professional attitude. Conduct respectful environment assessments immediately.

Practice Excellence

Perform thorough assessments and skilled interventions. Deliver patient-centered teaching.

Documentation Excellence

Create complete, accurate, survey-ready notes. Submit same day and align with POC requirements.



Compliance & Clinical Excellence: Module 2

Your license and our contracts depend on compliance. This module ensures every AAMS nurse practices legally, ethically, and at survey-ready quality. Understanding regulatory requirements is not optional—it's fundamental to safe, professional practice.

Understanding State & Federal Requirements

Federal Requirements

- Medicare Conditions of Participation (CoPs)
- HIPAA Privacy & Security Act
- OSHA safety guidelines
- CMS documentation standards
- CDC infection control protocols

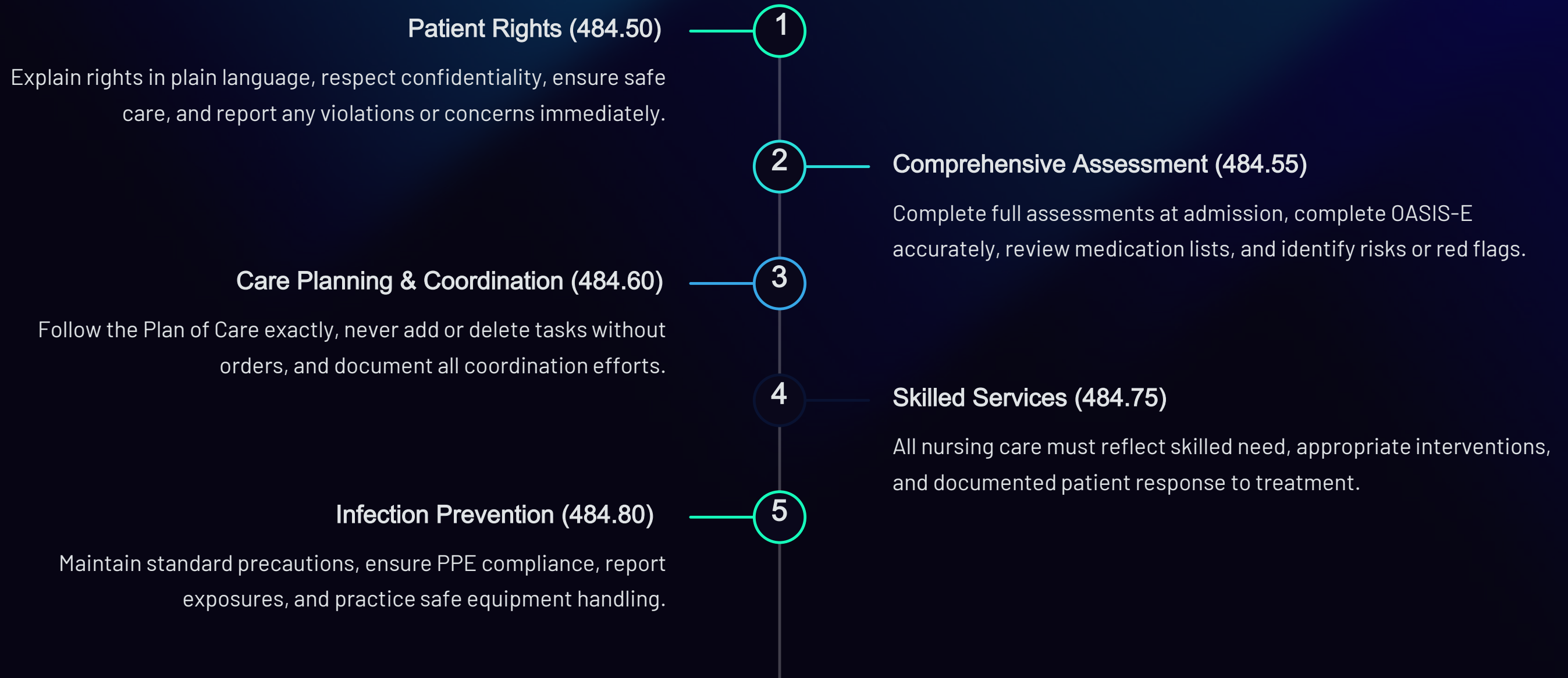
State Requirements

- Nurse Practice Act compliance
- Current licensing requirements
- Supervision rules (especially LVN/LPN)
- Documentation retention policies
- Mandated reporting obligations
- State survey and audit expectations

You must know your scope of practice, practice within your license limitations, and follow agency policies that reflect your state's regulations. Failure to comply puts your license, patient safety, and agency contracts at risk.

Medicare Conditions of Participation (CoPs)

The CoPs dictate how all home health agencies must operate. As an AAMS nurse, you must follow them because your documentation is part of the agency's compliance record and directly impacts survey outcomes.



Survey-Ready Documentation Standards

Your documentation must pass any audit—state, federal, or accreditation. Surveyors expect clear, objective, medically necessary documentation that supports the Plan of Care and demonstrates skilled nursing interventions.



Complete

No blank spaces or missing vital signs. Every required field must be accurately filled with appropriate clinical data.



Accurate

Reflects actual patient condition and tasks performed. Documentation must align with clinical findings and interventions.



Specific

Use measurable, descriptive language. Example: "Performed wound care per orders; wound size 2x3 cm; no drainage noted."



Consistent

Matches orders and POC across all documentation. Contradictions trigger survey deficiencies and compliance issues.



Objective

Avoid opinions or vague statements. Document observable facts and measurable patient responses to interventions.



Skilled

Describe interventions requiring nursing judgment, assessment skills, and clinical decision-making abilities.

Top Documentation Deficiencies & Prevention

Missing or Incomplete Vital Signs

Prevention: Always obtain vitals unless unsafe or refused. Document refusals clearly with patient's exact words and your clinical assessment.

Poor Medication Reconciliation

Prevention: Compare all sources including patient list, medication bottles, MAR, and hospital discharge records. Document OTC medications.

Missing Teaching Evidence

Prevention: Document what you taught, how you taught it, and the patient's demonstration of understanding or barriers to learning.

No Skilled Need Justification

Prevention: Describe specific interventions and assessments that demonstrate why the patient requires skilled nursing care.

Contradictory Notes

Prevention: Ensure clinical data aligns across all sections. Example: Don't document "no pain" and later write "pain interferes with ADLs."

OASIS Errors

Prevention: Read questions carefully, align responses with clinical findings, and ensure functional scoring accuracy reflects patient status.

Critical Escalation Red Flags

You MUST notify the agency AND AAMS immediately if you encounter any of the following situations. Delayed reporting can result in patient harm, regulatory violations, and contract termination.



Clinical Changes

Sudden change in condition, wound deterioration, falls, new or worsening confusion, or patient refusing critical care.



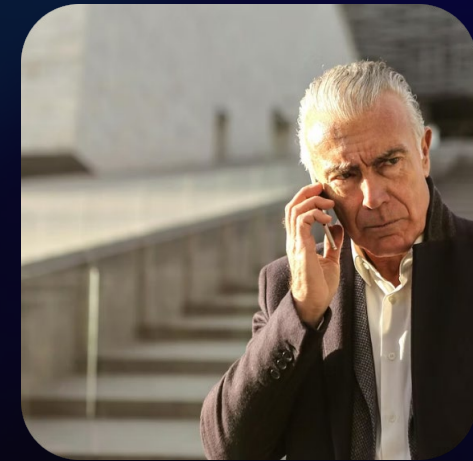
Medication Issues

Medication discrepancies between sources, adverse reactions, non-compliance, or dangerous drug interactions identified.



Safety Hazards

Unsafe home environment, new safety hazards identified, exposure to infectious disease, or threats to clinician safety.



Abuse or Neglect

Any concerns regarding abuse, neglect, or exploitation must be reported immediately per state mandated reporting laws.



Patient Visits & Professionalism

This comprehensive module ensures every AAMS nurse delivers safe, consistent, and survey-ready patient care from start to finish. Master these workflows to provide exceptional home health services that meet regulatory standards and exceed patient expectations.

Pre-Visit Workflow

What You Do BEFORE You Step Into the Home

A successful visit begins long before arriving at the patient's door. Thorough preparation ensures safety, compliance, and efficiency while setting the foundation for quality care delivery.

01

Review Orders & Plan of Care

Examine the POC for required tasks, confirm ordered frequency, check wound care orders and medication changes, and review last nurse notes for continuity.

03

Prepare Supplies & Equipment

Gather PPE, wound care supplies, vital sign equipment, documentation templates, and specialty items required for that specific patient.

02

Pre-Visit Communication

Call the patient 24 hours before to confirm appointment time, address, gate codes, and assess any new symptoms requiring escalation.

04

Personal Safety & Awareness

Assess surroundings before entering, park in visible safe areas, and maintain environmental awareness to ensure your personal safety.



Review Orders & Plan of Care

Review the POC for Required Tasks

Carefully examine all ordered interventions, assessments, and skilled nursing tasks that must be completed during the visit.

Confirm Ordered Frequency

Verify visit schedule matches physician orders (e.g., 3x/week, daily visits) to ensure compliance with the established care plan.

Check Wound Care Orders & Med Changes

Review specific wound care protocols, dressing types, and any recent medication adjustments or new prescriptions.

Review Last Nurse Notes

Read previous visit documentation to maintain continuity of care and understand recent patient status changes.

 **Red Flag:** If the POC does not match what the patient states, notify the agency immediately to resolve discrepancies before proceeding.

Pre-Visit Communication

Call the Patient 24 Hours Before

Proactive communication prevents missed visits and ensures smooth care delivery. When possible, contact patients a day in advance to:

- Confirm the appointment time and obtain verbal agreement
- Verify the address and obtain any gate codes or access instructions
- Ask about pets, caregiver availability, or special accommodation needs
- Assess any new symptoms that might require escalation to the physician

Best Practice

Always call again when you are en route (10–15 minutes away) to confirm the patient is home and ready for your arrival.



Prepare Supplies & Equipment



PPE

- Gloves (multiple sizes)
- Masks and face shields
- Gowns if needed for transmission precautions



Wound Care Supplies

- Appropriate dressings
- Cleansing solutions
- Tape and securing materials



Assessment Equipment

- BP cuff and stethoscope
- Thermometer
- Pulse oximeter



Documentation

- Visit templates and forms
- Patient education materials
- Specialty items for specific patients

Critical Preparation Tip: Always bring a backup option in case equipment fails. Battery-operated devices should have fresh batteries, and manual alternatives should be readily available.

Personal Safety & Awareness

1

Look for Unsafe Surroundings

Before exiting your vehicle, scan the environment for potential hazards, suspicious activity, or threatening situations.

2

Park in a Visible, Safe Area

Choose well-lit parking spots with clear exit routes. Ensure your vehicle is positioned for quick departure if needed.

3

Maintain Environmental Awareness

Stay alert to your surroundings, trust your instincts, and continuously assess the safety of the neighborhood environment.

✗ If You Feel Unsafe:

Do NOT enter the home. Your safety is the top priority. No visit is worth risking your personal wellbeing.

✓ Immediate Action:

Call the agency and AAMS to report safety concerns. Document the situation and follow organizational safety protocols.





During the Visit

Professional Conduct & Clinical Excellence

This is where professionalism meets clinical skill. Every action, assessment, and interaction reflects the AAMS standard of care and directly impacts patient outcomes and satisfaction.

Establishing Professional Presence



Knock and Wait for Acknowledgment

Never enter unannounced. Respect the patient's home by waiting for permission to enter.



Introduce Yourself Professionally

State your name, credentials, and the agency you represent clearly and confidently.



Show ID Badge

Display your identification prominently to establish credibility and reassure the patient.



Ask Permission Before Entering

Use respectful language: "May I come in?" This honors the patient's autonomy and dignity.



Maintain Warm, Calm, Respectful Demeanor

Your body language and tone set the foundation for a positive, therapeutic relationship.

Initial Observation

The First 10 Seconds Rule

Upon entering the home, conduct a rapid environmental assessment. These first impressions provide critical safety and clinical information that guides your entire visit.

What to Scan For:

- **Fall hazards:** cluttered walkways, loose rugs, poor lighting, unstable furniture
- **Odors:** signs indicating infection, wound issues, incontinence, or hygiene concerns
- **Signs of neglect:** unsafe living conditions, lack of food, utility shut-offs, or inadequate caregiving
- **Patient's immediate physical status:** distress, color, breathing pattern, mobility, mental clarity



Critical Action: Document concerning observations and escalate when appropriate. Early identification of safety issues prevents adverse events.



Safety & Infection Control



Hand Hygiene

Wash hands or sanitize immediately upon entering the home



Proper PPE

Use appropriate personal protective equipment based on patient condition



Precautions

Follow standard and transmission-based precautions consistently



Equipment Sanitation

Keep all equipment clean and sanitized between patients



Safe Removal

Remove gloves correctly to avoid cross-contamination

Clinical Assessment Based on POC

Perform Comprehensive Assessment

Every visit must include thorough evaluation aligned with physician orders and the established plan of care. Your clinical assessment forms the foundation for identifying changes and ensuring appropriate interventions.

Required Components:

- **Vitals:** blood pressure, heart rate, temperature, respiratory rate, oxygen saturation, pain level
- **Focused assessment:** cardiac, respiratory, GI, neurological, wound status, and other systems as indicated
- **Medication review:** compliance, effectiveness, side effects, and proper administration technique
- **Functional status check:** mobility, ADLs, safety awareness, and fall risk
- **New or worsening symptoms:** any changes since last visit that require clinical intervention



Communication During the Visit

Clear

Use straightforward language that leaves no room for misunderstanding. Avoid ambiguous terms.

Respectful

Honor the patient's dignity, autonomy, and personal preferences throughout all interactions.

Professional

Maintain appropriate boundaries and therapeutic relationships at all times.

Patient -Centered

Focus conversations on the patient's needs, concerns, and understanding of their care.

Use plain language and avoid medical jargon. Always confirm understanding by asking patients to explain information back to you in their own words.

Example: "Let's review your medication changes together to make sure everything matches what your doctor ordered. Can you tell me in your own words what you understand about when to take this new prescription?"

De-escalation & Conflict Prevention

In home health, situations can become emotional due to illness, stress, financial burden, or family dynamics. Use trauma-informed, professional strategies to maintain a safe, therapeutic environment.

- 1 Maintain a Calm, Low Tone**
Your voice sets the emotional temperature. Speak slowly and gently, even if the patient or caregiver is agitated.
- 2 Avoid Arguing**
Never engage in defensive or combative exchanges. Remain neutral and focused on finding solutions.
- 3 Give Space to Speak**
Allow the patient or caregiver to express concerns fully without interruption. Active listening defuses tension.
- 4 Acknowledge Feelings**
Validate emotions with empathetic statements: "I hear you, and I understand your concern. Let's work through this together."
- 5 Redirect Focus Toward Safety and Care**
Guide the conversation back to patient wellbeing and collaborative problem-solving.

 **If the environment becomes unsafe:** Remove yourself immediately, contact the agency and AAMS, and document the situation objectively. Your safety is non-negotiable.

Bedside Presence Standards



Professional Presence That Inspires Confidence

AAMS nurses must consistently demonstrate excellence in bedside manner. Your presence communicates competence, compassion, and commitment to quality care.

Essential Qualities:

- **Calmness:** Project confidence and composure, even in complex or urgent situations
- **Empathy:** Demonstrate genuine concern for patient wellbeing and emotional needs
- **Professional posture:** Maintain appropriate body language that conveys attentiveness
- **No rushing:** Allocate sufficient time to address all patient needs and concerns
- **Respect for boundaries:** Honor cultural, personal, and physical space preferences

Important: Your bedside presence heavily influences patient satisfaction scores and agency feedback. How you make patients feel directly impacts their perception of care quality and their willingness to engage in treatment plans.

HIPAA Compliance in the Home



Protect Patient Privacy in Conversations

Do not discuss patient details where others can overhear. Move to a private area if family members or visitors are present and the information is sensitive.



Secure All Documentation

Do not expose paper records or leave documents unsecured. Keep charts, forms, and printed materials in your possession at all times.



Use Secure Communication Platforms

Do not text PHI unless using a HIPAA-compliant, encrypted platform approved by your agency. Standard SMS is not secure.



Verify Authorization for Information Sharing

Confirm who is authorized to receive health information before discussing the patient's condition with family members or caregivers.

 **Critical Requirement:** HIPAA violations must be reported immediately to your agency and AAMS. Privacy breaches carry serious legal and professional consequences.

Post-Visit Workflow

How You End the Visit & Report Care

1

Summarize the Visit with the Patient

Recap what you did, what they need to follow up on, teaching provided, next scheduled visit, and red flags to watch for.

2

Communicate Changes or Concerns to Agency

Notify the agency same day if new symptoms, medication issues, safety concerns, or other significant changes occurred.

3

Complete Documentation

Submit your visit note the same day with comprehensive assessment findings, interventions, and plan for next visit.

4

Post-Visit EVV (Where Required)

Ensure proper clock in/out, accurate service codes, and report any system failures immediately.



Summarize the Visit with the Patient

Reinforce Understanding Before You Leave

Before departing, conduct a thorough recap with the patient and/or caregiver. This summary reduces confusion, improves compliance, and prevents hospitalizations by ensuring everyone understands the care plan.

Essential Elements to Cover:

- **What you did:** procedures performed, assessments completed, medications reviewed
- **Follow-up actions:** appointments to schedule, tests to complete, physician calls to make
- **Teaching provided:** disease management, medication changes, self-care techniques
- **Next scheduled visit:** confirm date, time, and what to expect
- **Red flags:** specific symptoms requiring immediate medical attention and when to call for help

This patient-centered approach reinforces understanding, builds confidence in self-management, and creates a safety net that reduces emergency department visits and hospital readmissions.



Communicate Changes or Concerns to Agency

You MUST notify the agency the same day if any of the following occurred during your visit. Timely communication ensures appropriate interventions and prevents adverse outcomes.

New or Worsening Symptoms		Medication Issues/Discrepancies
Safety Concerns		Refusal of Care
Signs of Abuse or Neglect		Wound Deterioration
Falls or Injury		Patient Hospitalization

Document Your Communication:

- Who you spoke with (name and title)
- Time of notification
- Summary of communication and recommendations made

 Clear, timely communication protects patients and demonstrates professional accountability.

Documentation Requirements

Same-Day Submission Required

Your visit is not complete until your documentation is submitted. Comprehensive, accurate, and timely documentation is essential for regulatory compliance, reimbursement, and continuity of care.

Assessment Findings

Document all vital signs, physical assessment findings, and clinical observations in detail.

Skilled Interventions Performed

Describe specific nursing interventions, procedures, and treatments provided during the visit.

Teaching and Patient Response

Record education provided and the patient's understanding, questions, and ability to demonstrate skills.

Progress Toward Goals

Evaluate patient advancement toward established care plan goals and outcomes.

Barriers or Safety Concerns

Note any obstacles to care delivery, compliance issues, or environmental safety hazards identified.

Coordination with Other Providers

Document communication with physicians, therapists, pharmacists, or other healthcare team members.

Plan for Next Visit

Outline the focus and anticipated interventions for the upcoming visit to ensure continuity.

 **Critical Warning:** Late notes can jeopardize agency contracts and AAMS relationships. Submit documentation the same day as the visit, every time.

Professionalism & Standards of Conduct

Behavioral Expectations

AAMS nurses must consistently demonstrate the highest standards of professional conduct. Your behavior reflects not only on yourself but on the entire organization.

Core Professional Values:

- **Accountability:** taking responsibility for actions, decisions, and outcomes
- **Respect for patient autonomy:** honoring patient rights and choices
- **Confidentiality:** protecting patient privacy and health information
- **Non-discriminatory behavior:** providing equitable care regardless of background
- **Professional communication:** clear, respectful, and appropriate in all interactions
- **Ethical decision-making:** upholding moral principles and nursing code of ethics

Boundaries & Ethical Practice

Do NOT:

- Accept gifts from patients or families
- Lend or borrow money
- Provide non-nursing services (cleaning, running errands)
- Share personal phone numbers unless required by agency policy
- Become involved in family conflicts or personal disputes

